3.12 Acute Respiratory Distress Syndrome (ARDS)*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Cannot speak in full sentences **Objective** • SpO_2 85% despite high-flow oxygen • Bilateral infiltrates on chest X-ray • Low PaO_2/FiO_2 ratio	**ACTUAL** Impaired gas exchange related to alveolar–capillary membrane injury as evidenced by refractory hypoxemia and bilateral infiltrates.	• Reaches the prescribed SpO_2 or PaO_2 target with ventilatory support.	• Follow ABG, SpO_2 and ventilator settings. • Assist with intubation and mechanical ventilation. • Place him prone according to protocol. • Maintain the prescribed PEEP; avoid needless circuit disconnection. • Give sedation as ordered.	• Considered met when oxygenation reaches target.
Subjective • Family say he was very anxious before intubation **Objective** • Fluid balance +3 L • Widespread bilateral crackles	**ACTUAL** Excessive fluid volume related to increased capillary permeability as evidenced by positive fluid balance and crackles.	• Fluid balance neutral or negative, as ordered.	• Keep a strict intake and output chart; weigh daily. • Give diuretics as prescribed. • Follow a conservative fluid strategy as ordered.	• Goal reached when fluid balance is on target.
Subjective *No specific complaint* **Objective** • Mechanically ventilated • Immobile; nursed prone	**RISK** Risk for adult pressure injury as evidenced by risk factors: immobility, sedation and prone positioning.	• Skin intact throughout ventilation and proning.	• Pad the face, chest, knees and other pressure points when prone. • Move the head and arms as the prone protocol directs. • Check skin under devices such as tube ties and masks every shift. • Use a pressure-redistributing mattress.	• Met if skin stays intact.

*References for this care plan: [19], [29], [35], [128]. Full entries are listed in the References section of the CPMN book.